Activity 7 — Converging: Impact-Ease Matrix and Weighted Decision Matrix

Meridian Health — choosing four from twenty

Topic	Topic 2
Objective	A5 · K4, K6 — Shortlist and evaluate the most viable ideas using decision-making techniques.
Duration	50 minutes
Grouping	Teams of 3-5
Tools	ChatGPT / Copilot / Gemini, Pinboard
Ed-tool	Pinboard — https://alfredang.github.io/pinboard/

The Scenario

You now hold twenty-plus candidate solutions from Activity 6. The Clinical Director will fund approximately four, inside S$250,000 over 12 months.

Two members of your team are already advocating loudly for their favourite ideas and the discussion is circling. This is exactly the moment where teams either make an evidence-based decision or default to whoever is most senior or most persistent.

Your job is to convert the board into a defensible shortlist using two complementary tools: a fast Impact-Ease screen, then a weighted decision matrix on the survivors.

The Evidence

Meridian Health — agreed decision criteria and weightings

Criterion	Weight	Scoring guidance (1-5)
Impact on root cause (retention/training)	35%	5 = directly fixes the root cause
Speed to measurable effect	20%	5 = effect visible within 8 weeks
Cost within S$250k envelope	20%	5 = under S$25k
Clinical/regulatory risk	15%	5 = no MOH implication at all
Staff and client acceptance	10%	5 = actively welcomed by both

Discussion Questions

1. Place all your ideas on the Impact-Ease matrix. Which quadrant is most crowded, and what does a crowded Fill-In quadrant tell you about how your team was thinking?
2. Run the weighted matrix. Did the top-scoring solution match your team's gut favourite from Activity 6? If not, what did the weighting expose?
3. Change the weight on 'Speed to measurable effect' from 20% to 40% and re-rank. Does the winner change? What does that sensitivity tell you about how robust your recommendation is?

4. The AI recommended a portfolio of four. Explain why the best four INDIVIDUAL scores are not automatically the best four TOGETHER.
5. Consensus fatigue is when a team accepts a weak option just to end the discussion. Where in this activity was your team most at risk of it, and what stopped you?

Step-by-Step

6. Bring the full idea board from Activity 6. Remove any duplicates by merging them.
7. Draw the Impact-Ease matrix on a flip chart with four labelled quadrants.
8. Place every idea in a quadrant as a team. Where you disagree, place it on the line and move on — do not stall.
9. Discard the Thankless Tasks. Set the Fill-Ins aside as 'do if free capacity'.
10. BEFORE scoring anything, agree the five criteria weights as a team. Write them down. This prevents arguing about scores when you actually disagree about weights.
11. Build the weighted decision matrix for the surviving Quick Wins and Big Bets. Score each criterion 1-5.
12. Compute the weighted totals and rank.
13. Run the GenAI convergence prompt from the slide with your actual list pasted in.
14. Compare the AI ranking to yours. Investigate every place they disagree by more than two positions.
15. SENSITIVITY TEST: change the speed weight from 20% to 40% and re-rank. Record whether the winner changes.
16. Select your final FOUR as a PORTFOLIO — check they do not all attack the same cause, and check for redundancy.
17. Prepare a two-minute recommendation to the Clinical Director: the four, the total cost, the criteria, and the one thing you are still leaving unaddressed.

The GenAI Prompt

```
Act as a decision analyst. I will give you a list of candidate solutions. Do NOT add
new ideas.

STEP 1 — Classify every solution on an Impact vs Ease matrix as exactly one of:
- Quick Win (high impact, easy)
- Big Bet (high impact, hard)
- Fill-In (low impact, easy)
- Thankless Task (low impact, hard)
Give a one-line reason for each placement.

STEP 2 — Take only the Quick Wins and Big Bets and score them in a weighted decision
matrix:
- Impact on root cause (weight 35%)
- Speed to measurable effect (20%)
- Cost within a S$250k envelope (20%)
- Clinical/regulatory risk (15%)
- Staff and client acceptance (10%)
Score each 1-5, show the weighted total, and rank them.

STEP 3 — Recommend the best FOUR as a portfolio, and explicitly state:
- why this combination is stronger than the top four individual scores
- which two solutions overlap or make each other redundant
```

```
- what the portfolio still leaves unaddressed

Solutions: [paste your team's list here]
```

Trainer Debrief

Expected answers and the points to draw out. Trainer reference.

EXPECTED SHAPE OF A STRONG ANSWER:
QUICK WINS — roster all 5 permanent phlebotomists at 07:00 and taper later (near-zero cost, immediate effect); early-shift differential pay; pre-visit fasting-compliance reminders; route difficult draws to trained staff.
BIG BETS — structured 10-day onboarding with competency sign-off; convert top relief staff to permanent; corporate on-site draws.
FILL-INS — better signage, updated leaflets, minor booking-form changes.
THANKLESS — a new centre; a full HR system replacement.

A defensible portfolio of four usually pairs an immediate stabiliser with a root-cause fix, e.g.: (1) re-roster the 07:00 shift — buys relief NOW at almost no cost; (2) early-shift differential — attacks why people leave the early shift; (3) structured onboarding with competency sign-off — attacks the 14-min relief draw time and the 18% repeat rate; (4) convert top relief staff to permanent — converts the agency pool into retained capacity.

KEY TEACHING POINTS:
1. TWO TOOLS, TWO JOBS. Impact-Ease is a fast screen that clears the board of Fill-Ins and Thankless Tasks in minutes. The weighted matrix is the rigorous instrument you use on the survivors and, crucially, the one you can DEFEND to a board. Using the heavy tool on all twenty wastes the session; using only the light tool leaves you unable to justify the call.
2. THE WEIGHTS ARE THE REAL DECISION. Most teams argue about scores when the disagreement is actually about weights. Surfacing weights first — before any scoring — converts a political argument into an explicit, recorded choice. This is the single most useful facilitation move in the activity.
3. SENSITIVITY TESTING: if doubling the speed weight flips the winner, your recommendation is fragile and you must say so. If the same solution wins under several weightings, you have a robust recommendation. Boards trust the second kind. Teach learners to run the test BEFORE they present, not after they are challenged.
4. PORTFOLIO, NOT LEADERBOARD: the top four scores may all attack the same cause and leave another exposed, or two may be redundant (on-site corporate draws largely removes the need for the 07:00 re-roster for those clients). A portfolio balances quick stabilisation against durable root-cause repair. This is where human judgement outperforms the AI's ranking, and learners should see that clearly.
5. AI LIMITATION TO NAME OUT LOUD: the AI will confidently invent cost figures and timelines it has no basis for. Its ranking is only as good as the criteria you set. It is a fast, tireless, unbiased-by-politics scorer — not a decision maker. The Clinical Director will ask YOU why, not the model.

Self-Check — Is Your Output Finished?

Your shortlist is defensible when: every surviving idea sits in a named quadrant; the weighted matrix shows criteria, weights, scores and totals; you have run at least one sensitivity test and can state whether the winner held; your four are a portfolio rather than the top four scores; the total cost fits inside S$250k; and you can name one thing the portfolio does NOT fix. If you cannot explain to the Clinical Director why idea #5 lost, the matrix has not done its job.